

1. Administrative & Study Identification

Full Study Title: A Phase III, Randomized, Open-Label, Multicenter Study Evaluating the Efficacy and Safety of Divarasib Versus Sotorasib or Adagrasib in Patients With Previously Treated KRAS G12C-Positive Advanced or Metastatic Non-Small Cell Lung Cancer **Short Title/Acronym:** Krascendo 1 **Protocol Number:** B045217 **NCT Number:** NCT06497556 **Sponsor Name:** Hoffmann-La Roche (Roche) **Investigational Product:** Divarasib (Active Comparators: Sotorasib or Adagrasib) **Phase of Development:** Phase III **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the safety and efficacy of divarasib compared to locally approved KRAS G12C inhibitors (sotorasib or adagrasib) as measured by Progression-Free Survival (PFS). **Secondary Objectives:** To evaluate Overall Survival (OS), Confirmed Objective Response, Duration of Response (DOR), Disease Control Rate (DCR), Time to Confirmed Deterioration (TTCD) on quality of life questionnaires, and the overall safety profile of the treatments.

2.2 Background & Rationale Non-small cell lung cancer (NSCLC) is the most common type of lung cancer. In a subset of patients, tumors harbor a change in the KRAS gene (KRAS G12C-positive), causing rapid cancer cell growth and survival. The study aims to evaluate if divarasib, a next-generation KRAS G12C inhibitor, offers superior efficacy and/or safety compared to the currently available standard-of-care targeted therapies (sotorasib or adagrasib) for patients with previously treated advanced or metastatic KRAS G12C-positive NSCLC whose cancer has spread or who have no other standard treatments available.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Sotorasib or Adagrasib) **Blinding:** Open-label (None) **Randomization:** Randomized (Parallel Assignment) **Trial Status:** RECRUITING

3.2 Planned Enrollment & Duration Target \$N\$: 320 participants **Number of Sites:** Approximately 157 locations across 26 countries **Estimated Study Start:** 23-Sep-2024 **Estimated Primary Completion:** 30-Sep-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Unequivocal histologically or cytologically confirmed diagnosis of metastatic or locally advanced NSCLC not amenable to treatment with surgical resection or combined chemoradiation. 2. Disease progression during or after treatment with at least one prior systemic therapy but no more than three lines of prior systemic therapy in the advanced or metastatic setting. 3. Measurable disease according to Response Evaluation Criteria in Solid Tumors (RECIST) v1.1. 4. Documentation of the presence of a KRAS G12C mutation. 5. Availability of a representative formalin-fixed, paraffin-embedded (FFPE) tumor specimen in a paraffin block (preferred) or 10-15 (15 preferred) unstained, freshly cut, serial slides with an associated pathology report. 6. Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1. 7. Life expectancy of ≥ 12 weeks.

4.2 Exclusion Criteria 1. Known hypersensitivity to any of the components of divarasib, or sotorasib or adagrasib. 2. Known and untreated, or active central nervous system (CNS) metastases, leptomeningeal disease, or carcinomatous meningitis. 3. Prior treatment with any KRAS G12C inhibitor or pan-KRAS/RAS inhibitor. 4. Mixed small-cell lung cancer or large cell neuroendocrine histology. 5. Malabsorption syndrome or other condition that would interfere with enteral absorption. 6. Known concomitant second oncogenic driver. 7. Uncontrolled pleural effusion, pericardial effusion, or ascites requiring recurrent drainage procedures biweekly or more frequently. 8. Any infection that, in the opinion of the investigator, could impact patient safety, or treatment with therapeutic oral or IV antibiotics within 14 days prior to Day 1 of Cycle 1. 9. More than 30 Gy of radiotherapy to the lung within 6 months of randomization. 10. Uncontrolled tumor-related pain. 11. Unresolved toxicities from prior anticancer therapy. 12. History of malignancy within 5 years prior to screening, with the exception of the cancer under investigation in this study and malignancies with a negligible risk of metastasis or death (e.g., 5-year OS rate $>90\%$), such as adequately treated carcinoma in situ of the cervix, nonmelanoma skin carcinoma, localized prostate cancer, ductal carcinoma in situ, or Stage I uterine cancer.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Divarasib orally, once daily (QD). **Route of Administration:** Oral. **Duration of Treatment:** Until disease progression, unacceptable toxicity, or patient withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care measures as per protocol guidelines. **Prohibited:** Prior treatment with any KRAS G12C inhibitor or pan-KRAS/RAS

inhibitor; therapeutic oral or IV antibiotics within 14 days prior to Day 1 of Cycle 1; more than 30 Gy of radiotherapy to the lung within 6 months of randomization.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor biopsy/slides (KRAS G12C confirmation), Baseline scans
Baseline	Day 0	Randomization, First Dose
Interim	Every 3 weeks	Treatment administration, Safety Labs, Efficacy Assessment, AE monitoring (approx. 4 visits during the first 6 weeks)
End of Study	Follow-up	Tumor assessment every 6 weeks until disease progression, then survival follow-up every 3 months

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS): Time from randomization to the first occurrence of disease progression, as determined by blinded independent central review (BICR) according to RECIST v1.1, or death from any cause (whichever occurs first) (up to approximately 4 years).

Measurement Method: Blinded independent central review (BICR) according to RECIST v1.1.

7.2 Secondary & Exploratory Endpoints * Overall Survival (OS) * Confirmed Objective Response (Objective Response Rate - ORR) * Time to Confirmed Deterioration (TTCD) on the European Organisation for Research and Treatment of Cancer Quality of Life Questionnaire (EORTC QLQ-C30) Dyspnea Item and Physical Functioning Scale * Duration of Response (DOR) * Disease Control Rate (DCR)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of unwanted effects (mild to severe) during regular check-up visits every 3 weeks. **Serious Adverse Events (SAEs):** Reported continuously throughout the active study period and safety follow-up. **Data Monitoring Committee (DMC):** Utilized for ongoing safety and efficacy review as per Sponsor's standard oncology trial procedures.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) set for efficacy endpoints; Safety Analysis Set for safety parameters. **Sample Size**

Justification: Target enrollment of 320 patients powered for PFS comparison between divarasib and active comparators. **Handling of Missing Data:** Censoring applied for PFS and OS calculations as per standard time-to-event methodology (e.g., Kaplan-Meier).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring of trial sites (157 locations globally) to ensure protocol compliance.

Audit Trail: Electronic Data Capture (EDC) systems utilized with full eCRF tracking and data clarification processes.

11. Study Identifiers & Governance

Primary ID: B045217 **Registry Number:** NCT06497556 **Sponsor/Lead Organization:** Hoffmann-La Roche (Roche) **Study Title:** Krascendo 1 **Official Regulatory Title:** A Phase III, Randomized, Open-Label, Multicenter Study Evaluating the Efficacy and Safety of Divarasib Versus Sotorasib or Adagrasib in Patients With Previously Treated KRAS G12C-Positive Advanced or Metastatic Non-Small Cell Lung Cancer

12. Clinical Framework (Oncology Specific)

Primary Condition: Non-Small Cell Lung Cancer (NSCLC), KRAS G12C Lung Cancer **Diagnosis Threshold:** KRAS G12C mutation positive, previously treated **Medical Methodology:** Targeted Therapy (KRAS G12C inhibitor) **Optimization Target:** Disease control and progression-free survival (PFS)

13. Study Procedures & Methodology

Management Pathway: Advanced/Metastatic NSCLC pathway post-prior systemic therapy **Education Protocol (HCP):** Investigator training on administration and toxicity management of Divarasib

Education Protocol (Patient): Informed consent detailing experimental nature of divarasib, possible adverse effects, and visit schedule

Quality Audit Mechanism: Centralized RECIST v1.1 imaging review via BICR

14. Observation & Follow-Up Schedule

Total Duration: Up to approximately 4 years **Onsite Visit Cadence:** Approximately 4 times during the first 6 weeks, then every 3 weeks during treatment **Remote Monitoring Frequency:** Survival

follow-up every 3 months after disease progression **Checklist**
Review Interval: Tumor assessment (imaging) every 6 weeks

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				